STIs ON LABOR AND DELIVERY: WHAT TO DO NOW

FD-ANT-STI · Companion to STIs on Labor and Delivery · CDC STI Treatment Guidelines 2021 · ACOG PB 220 · Labor and delivery / OBED · Decision support only, not unit policy

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HOW TO READ THIS CHART: start by choosing **In labor / admitted** or **OB ED triage**. The labor path is the router below (hard stops first). The OB ED triage branch is for patients not in labor or postpartum ≤12 wk. Doses and regimens live on the companion card; this chart lists none.

SHADING KEY: WHITE = can wait / no delivery-mode change · LIGHT GRAY = admission check · MID GRAY = treat before discharge · DARK GRAY = hard stop — changes delivery

START: IN LABOR / ADMITTED, OR OB ED TRIAGE?

Choose the path that matches how she presents.

IN LABOR / ADMITTED · WHITE
Continue to the labor router (nodes 1–5 below).

OB ED TRIAGE · WHITE
Not in labor (any GA) or postpartum ≤12 wk. Continue to the OB ED triage branch below.

1. ADMISSION STATUS CHECK · LIGHT GRAY
Syphilis, HIV and HBsAg must be known; also GC/CT and HSV history. Anything unknown gets ordered now. Then go to node 2.

2. HARD STOPS — SHOWN FIRST

Does anything change how she delivers? Check both boxes. Either or both may apply.

ACTIVE GENITAL HSV · DARK GRAY
Active genital HSV lesions or prodrome, or ROM with lesions: cesarean. This is a hard stop. See the companion card for the rest of HSV management.

HIV · DARK GRAY
HIV reactive or unknown with risk, or viral load over 1,000 or unknown: IV ZDV, then the HIV card / HHS guidance for mode of delivery. This is a hard stop.

3. TREAT BEFORE DISCHARGE — DELIVERY MODE UNCHANGED · MID GRAY
These do not change how she delivers. Treat before she leaves. Doses and regimens are on the companion card.

SYPHILIS · MID GRAY
Positive or undocumented, including discordant results: benzathine penicillin by stage. Desensitize if she is allergic. Peds handoff with her titers and treatment dates. (Doses on the card.)

GC OR CT · MID GRAY
Positive or untreated: treatment per the card. Tell peds about untreated GC. Order EPT.

4. CAN WAIT UNTIL POSTPARTUM OR ROUTINE CARE · WHITE
Trich, symptomatic BV, yeast, test of cure, and suppression counseling. **Fluconazole only after delivery.** Regimens are on the companion card.

5. PEDS HANDOFF SUMMARY · LIGHT GRAY
Auto-list whatever was flagged: syphilis; HSV (primary vs recurrent); untreated GC; HBsAg status; and HIV. Use the peds-handoff cue with this algorithm. Nothing is stored.

OB ED TRIAGE BRANCH · LIGHT GRAY

Not in labor (any GA) or postpartum ≤12 wk. Early pregnancy, location unknown, or pain/bleeding <20 wk: confirm pregnancy and location first; ectopic and septic abortion outrank every STI diagnosis.

UNIVERSAL FIRST STEP · LIGHT GRAY
GA/viability and FHT or EFM. Vitals and sepsis screen. Last prenatal labs (syphilis, HIV, HBsAg, GC/CT, urine culture) — draw any that are missing. Ask about partners, prior STI, and social risk. Contractions, bleeding, ROM or abnormal FHR → L&D;, not this pathway.

a) DISCHARGE / ITCHING · WHITE
Workup and treatment per the card. Trich: always treat, including an incidental asymptomatic finding. BV symptomatic only. VVC: no oral fluconazole in pregnancy.

b) LESIONS / ULCERS · WHITE
HSV PCR, syphilis serology, HIV. Darkfield/PCR if available (do not wait). Treatment per the card.

c) PELVIC PAIN / FEVER · WHITE
Rule out ectopic, septic abortion, IAI, pyelo first. PID in pregnancy: hospitalize, IV, ID consult. Regimen per ID/pharmacy.

d) DYSURIA · WHITE
UA, culture, and GC/CT/trich NAAT. Pyelonephritis: admit as an inpatient for IV antibiotics. Cystitis/ASB: treatment per the card.

e) SYPHILIS SCREEN · WHITE
Treat in OBED; do not send her home for the shot. House practice (not a cited rule): J-H — before ~24 wk, discharge with return precautions; from 24 wk, first early-syphilis dose on L&D; with fetal monitoring for several hours.

f) PARTNER / EPT g) HIV OR HEP B UNKNOWN · WHITE
Test and treat exposures per the card. EPT permitted in Texas for GC/CT, not syphilis. Unknown HIV: opt-out test, link to care. Unknown hep B: SMFM #69 — newborn vaccine plus HBIG within 12 h.

BEFORE DISCHARGE · LIGHT GRAY
Single-dose treatments given in OBED, not just prescribed. Doses live only on the companion card. TOC/retest, Texas reporting, partner treatment or DIS, prenatal follow-up within 1 wk. Return precautions.